INFORMATION TO KEEP WITH YOU

My complete treatment handbook

AT 50/75

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00423
Version 5

> Doxorubicin followed by docetaxel on Day 1 every 21 days, for up to six cycles unless progression or unacceptable toxicity changes the plan.

Treatment stages

01 **AT 50/75 · treatment cycles**
21-day interval

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

AT 50/75
National Cancer Control Programme (NCCP) 00423

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Seek prompt advice for fever, shivering, severe diarrhoea, painful mouth ulcers, inability to drink, new chest symptoms or a painful/leaking line. Do not wait for the next cycle.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

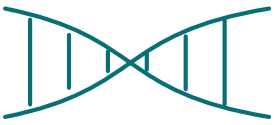
This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How the medicines work

Treatment to control cancer and symptoms, with regular review of benefit and side effects.

ANTHRACYCLINE CHEMOTHERAPY

Doxorubicin



Interferes with deoxyribonucleic acid (the cell's genetic material) (DNA) handling

Doxorubicin interferes with enzymes and DNA processes needed for cell division. It can also affect heart muscle, so the team considers previous exposure and cardiac risk.

DNA handling disrupted → Cell damage → Division reduced

What to watch for

Report pain or burning at the infusion site immediately. New breathlessness or chest symptoms need prompt review. Red urine for a short time can be a drug-colour effect; pain, clots or persistent bleeding still need advice.

TAXANE CHEMOTHERAPY

Docetaxel



Disrupts the cell's division machinery

Docetaxel stabilises tiny structures called microtubules. Cells need these to separate their chromosomes during division; disrupting the process can stop cancer cells multiplying. Healthy dividing cells are also affected.

Microtubules stabilised → Division disrupted → Cell growth affected

What to watch for

Take prescribed steroid premedication. Report fever, lasting numbness, swelling, sore nails or infusion symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

AT combines an anthracycline with a taxane when combination chemotherapy is appropriate for locally advanced or metastatic disease. Cancer biology, symptoms, need for tumour shrinkage, previous treatment, fitness, heart health, liver tests and existing nerve symptoms affect suitability.

Your treatment setting

Locally advanced or metastatic breast cancer

The sequence

Doxorubicin followed by docetaxel on Day 1 every 21 days, for up to six cycles unless progression or unacceptable toxicity changes the plan.

Alternatives and your priorities

Ask what alternatives fit your cancer, what the treatment aims to achieve, and how its practical demands fit your life. The team can discuss personal benefits and risks.

Why this treatment may be chosen

Show trial results and what the numbers mean

Locally advanced or metastatic breast cancer

- Pembrolizumab + chemotherapy
- Placebo + chemotherapy

Your team can discuss expected benefit and alternatives in the context of your own cancer and priorities.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

AT 50/75
NCCP 00423

The phases and how they connect

AT 50/75 · a reference 21-day cycle

21-day view



Treatment · Day 1
Day 1

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY INTERVAL

AT 50/75 · treatment cycles

21-day interval

Doxorubicin followed by docetaxel on Day 1 every 21 days, for up to six cycles unless progression or unacceptable toxicity changes the plan.

Docetaxel steroid tablets start the day before treatment. The team confirms whether the aim is local control or control of metastatic disease.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. The clinical team owns booking, result review and action on overdue tests. This is an educational map, not an automated appointment or result tracker.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00423 v5 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

AT 50/75: treatment and checks

Locally advanced or metastatic breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Cancer medicines NCCP 00423	Agree aim, indication and medicines	AT 50/75 starts	Review response, toxicity and exposure	Complete planned course / next decision	Ongoing review / subsequent plan

Care / check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Surgery METASTATIC	multidisciplinary team (MDT)-selected local treatment	No automatic operation during cycles	Review response / surgical plan	Selected for local need	Pathology and recovery guide next steps
Radiotherapy EARLY	Assess indication	Usually after cytotoxic chemotherapy	Arrange sequential timing	Radiation team confirms start	Review effects / follow-up
Blood tests NCCP 00423	full blood count (FBC), kidney and liver profile	Review before chemotherapy cycles	Check again at any phase change	Follow active medicines and abnormalities	Follow unresolved abnormalities
Echo / heart NCCP 00423 · ESC	Baseline echo (ESC); assess risk	High cardiac risk: repeat after 2 anthracycline cycles	If continuing: repeat after 4 cycles at high risk	Review cardiac symptoms and total lifetime exposure	Echo within 12 months; high risk within 3 months
electrocardiogram (ECG) NCCP 00423	Baseline ECG in the NCCP anthracycline source; repeat for symptoms or clinical risk. An ECG does not replace an echo.	Symptoms / risk-based repeat	Review if cardiac concern	ECG and echo answer different questions	Clinical indication
Cancer imaging NCCP 00423	Record baseline extent as appropriate	Clinical response / symptoms review	Metastatic: review about2–4 months	Arrange selected surgery / next treatment	Metastatic response plan continues
Markers / bone health NCCP 00423	Tests selected for active medicines	No routine tumour-marker clearance	Markers only as selected adjunct in metastatic care	dual-energy X-ray absorptiometry (bone density) (DEXA) if endocrine/ bone-risk indication	Follow the endocrine and bone-health plan

Care / check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Endocrine / bone BONE · EARLY	Confirm hormone receptors	Plan endocrine therapy when appropriate	Separate from this chemotherapy prescription	Start/continue per MDT; DEXA when indicated	Risk-based bone review

What each check is for

1 Before first dose

Baseline

Baseline FBC, kidney and liver tests; regular tests are required by NCCP. The unit should confirm pre-cycle timing and review results before prescribing.

Ask / act: Confirm tests and consent before prescribing.

2 Heart-monitoring plan

Baseline → cycles → after treatment

ESC supplementary guidance: baseline echo before anthracycline treatment. High/very-high cardiac risk: echo every two anthracycline cycles (for example before cycles 3 and 5 when continuing), and within 3 months after completion. All adults: echo within 12 months after completion. Other repeat tests depend on risk, cumulative exposure and symptoms.

Ask / act: Name the team responsible for booking and reviewing results.

3

Treatment-phase review

Before each transition

Doxorubicin followed by docetaxel on Day 1 every 21 days, for up to six cycles unless progression or unacceptable toxicity changes the plan.

Ask / act: Confirm active medicines, support script and the next test checkpoint.

4 Response and follow-up

Planned review points

Record disease sites before treatment. Response imaging is generally considered every 2–4 months under ESMO guidance, adjusted to symptoms and disease pace. For a 21-day course this is approximately after 3–6 cycles; the team chooses the scan and exact checkpoint. New symptoms may need earlier assessment.

Ask / act: Review symptoms and imaging together; do not change treatment on tumour markers alone.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before the first visit

Blood tests, relevant cardiac assessment, medicines review, fertility discussion and the consent conversation come first.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Staff check the line, give prescribed premedicines and treatment, and observe for reactions. Tell them immediately about burning, pain, rash or breathlessness.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

Allow time for blood review, preparation, treatment and observation. Your unit confirms the full visit duration. Arrange transport or an escort where advised.

Phase-specific visit pattern

These medicines are given on Day 1 in the unit. No take-home infusion pump is part of this regimen; port needle removal after a visit is different from removal of the implanted port.

Checks across treatment

Your team arranges, reviews and acts on tests; an overdue result needs follow-up.

Blood tests

Baseline FBC, kidney and liver tests; regular tests are required by NCCP. The unit should confirm pre-cycle timing and review results before prescribing.

Heart monitoring

ESC supplementary guidance: baseline echo before anthracycline treatment. High/very-high cardiac risk: echo every two anthracycline cycles (for example before cycles 3 and 5 when continuing), and within 3 months after completion. All adults: echo within 12 months after completion. Other repeat tests depend on risk, cumulative exposure and symptoms.

ECG

Baseline ECG in the NCCP anthracycline source; repeat for symptoms or clinical risk. An ECG does not replace an echo.

Cancer scans

Record disease sites before treatment. Response imaging is generally considered every 2–4 months under ESMO guidance, adjusted to symptoms and disease pace. For a 21-day course this is approximately after 3–6 cycles; the team chooses the scan and exact checkpoint. New symptoms may need earlier assessment.

Surgery, radiotherapy and later treatment

In metastatic disease, surgery and radiotherapy are individual decisions for symptoms or selected local-control needs; they are not automatic steps after every course.

Hormone-receptor-positive disease

Endocrine treatment is considered when appropriate for the cancer biology and setting. It has its own schedule and monitoring; it is not automatically part of this chemotherapy prescription.

Bone health

DEXA is linked to an aromatase inhibitor, ovarian suppression, treatment-induced menopause or other bone risk—not automatically to every chemotherapy course. The endocrine team confirms baseline testing and risk-based repeat scans.

Markers and follow-up

Tumour markers can support selected metastatic assessments but must not alone determine progression or treatment changes.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

AT 50/75
NCCP 00423

AT 50/75 · a reference 21-day cycle

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment · Day 1	 **Day 1** Medicines given in the unit
Nausea / appetite	 **Days 1–5** Often early; can last longer
Blood counts / infection	 **Days 1–21** Timing varies; fever needs immediate advice
Mouth / bowel	 **Days 1–21** Report ulcers, diarrhoea or poor intake
Fatigue	 **Days 1–21** May build over repeated cycles
Nerves / nails	 **Days 1–21** May accumulate with docetaxel

General orientation: Effects can occur at any time or continue beyond the chart. There are no guaranteed symptom-free days.

No dates to enter: Your team books appointments and reviews results. The graph requires no personal information.

> **Why these bands stay broad:** These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00423 v5 · official regimen ↗

There are no guaranteed “safe days”. Fever, shivers, sudden illness, severe breathing changes or chest pain need prompt assessment on any day.

Between-treatment notes

Day 1–21: A reference treatment cycle

Doxorubicin followed by docetaxel on Day 1 every 21 days, for up to six cycles unless progression or unacceptable toxicity changes the plan. Follow the written appointment and medicine plan; no date entry is needed to use this chart.

Support around the medicines

1 

Steroid tablets start before the visit

Follow the docetaxel steroid prescription from the day before treatment. If doses are missed, call the unit before attending; do not improvise a replacement.

2

A clear anti-sickness plan

Take the prescribed prevention medicines and keep the rescue instructions. Contact the team if nausea or vomiting prevents drinking.

3

Infection and growth-factor plan

The team decides whether growth-factor injections are needed and gives the exact days. Fever or suddenly feeling unwell needs immediate advice.

4

Hydration and urinary symptoms

Report new swelling, breathlessness or rapid weight gain. Do not change prescribed steroids yourself.

5

Before treatment

Discuss fertility, contraception, other medicines and line access. Tell staff immediately about burning or pain at the infusion site.

Questions worth asking

Who arranges the scans and tests?

Your treating service should book the planned checks, review the results and explain any change. If an expected appointment is missing, contact the team; this map does not book tests or confirm that they have happened.

Try explaining the plan back: Can you explain which medicines you are receiving now? What symptoms mean you should call urgently? Which blood tests, scans or heart checks come next, and who arranges them?

Doxorubicin

- Low counts, infection and mouth ulcers
- Nausea, hair loss and fatigue
- Heart damage, sometimes delayed
- Severe tissue injury if the infusion leaks
- Rare later blood cancers

Docetaxel

- Low blood counts, infection and sepsis
- Hair loss, mouth soreness and fatigue
- Numbness / tingling; sometimes lasting
- Fluid retention and nail changes
- Allergic infusion reaction

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

AT 50/75 · treatment-day support

Active medicines: Doxorubicin + Docetaxel

Emetogenic risk: Moderate · doxorubicin50 with docetaxel

Medicine	Dose / route	When
Ondansetron	8 mg orally twice daily	Day 1 NCCP alternative to 16 mg once daily; review QT / hepatic risks
Dexamethasone	8 mg orally	Day 1; reconcile docetaxel premedication Do not add to the docetaxel steroid course automatically
Dexamethasone	8 mg orally daily	Days 2–3 reference antiemetic row Reconcile the delayed period with docetaxel steroids; one final individualised steroid prescription
Metoclopramide	10 mg orally up to three times daily as required	Subsequent days: duration to be confirmed Review contraindications and renal function

Additional phase-specific support

- Docetaxel: dexamethasone 8 mg orally twice daily for 3 days starting the day before chemotherapy; at least 3 doses before the infusion. If missed, contact the team; any intravenous (IV) replacement is a consultant decision.
- Reconcile the total dexamethasone schedule with antiemetics, including aprepitant interactions. The listed antiemetic steroid is a reference row, not an instruction to add a second course.
- Assess granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) according to the exact regimen and individual risk.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

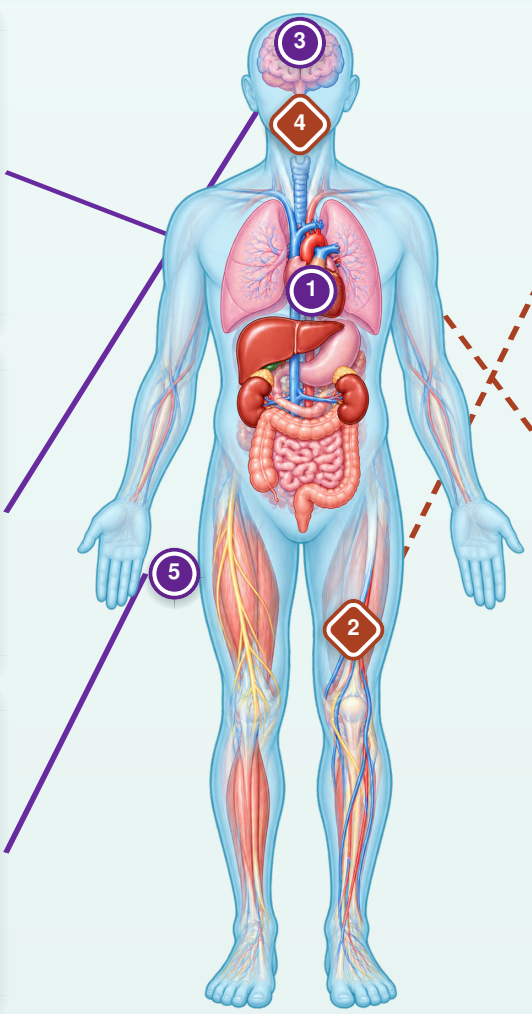
AT 50/75

Doxorubicin followed by docetaxel on Day 1 every 21 days, for up to six cycles unless progression or unacceptable toxicity changes the plan.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Heart

Doxorubicin

Baseline assessment; report chest pain, breathlessness or swelling.

2 Blood / infection

Chemotherapy phase

Fever, shivers or suddenly unwell: call immediately.

3 Hair / scalp

Chemotherapy phase

Hair loss is common. Ask about scalp cooling before treatment where suitable.

4 Mouth

Chemotherapy phase

Painful ulcers or difficulty drinking need advice.

5 Nerves / nails

Docetaxel

Report numbness, tingling or painful nails before the next dose.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Selected medicine-specific effects are highlighted. The map is not a complete list; persistent or new symptoms need review even after a medicine stops.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

AT 50/75
National Cancer Control Programme (NCCP) 00423

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **Regimen selection and source references**
   https://healthservice.hse.ie/documents/6823/423_DOXO_DOCEtaxel_AT_50_75-21day.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
4. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
5. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
6. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
7. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
8. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
9. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
10. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
11. **ASCO · biomarkers in metastatic breast cancer**
    https://ascopubs.org/doi/10.1200/JCO.2015.61.1459